

TOSCANA DIAGNOSTICA S.r.l.

Sistema di Gestione Integrato

FIRST TRIMESTER COMBINED SCREENING REQUEST ENGLISH VERSION OF TD-SGI-DEX010

Area 17 – Accettazione

Codice	Revisione	Data	Pagine
TD-SGI-DEX013_00	00	28/09/2026	3
Redatto da	Approvato da	Classificazione	Stato
RSGI	Direzione Generale	Interno	In vigore

Responsabile del Documento	RSGI	Emissione del Documento
Gian Luigi Taddei	Iacopo Cupelli	Francesco Epifani
28/09/2026	28/09/2026	28/09/2026

Versione in lingua inglese di TD-SGI-DEX010_00 (Richiesta Screening Combinato di 1° Trimestre – modulo del laboratorio in service), destinata alle pazienti e ai medici che non comprendono la lingua italiana. Il contenuto corrisponde integralmente al modulo del fornitore adottato con DEX010; in caso di dubbio interpretativo prevale la versione italiana. Rif.: TD-SGI-DEX010 · TD-SGI-INF067 (Informativa Test Combinato) · TD-SGI-PRO040 (Comunicazione con il Paziente e Consenso Informato) · TD-SGI-PRO042 (Gestione Fase Preanalitica).

Toscana Diagnostica S.r.l.

Via di Pratignone, 13/4, 50019 Sesto Fiorentino (FI). Tel. e Fax: 055776511. Direttore Sanitario: prof. Gian Luigi Taddei.
P.IVA e C.F.: 07528130482, REA n. FI-709602 – Capitale Sociale: € 50.000,00 i.v.

First trimester combined screening request

PERSONAL DETAILS

Name: _____ Surname: _____
 Date of birth: _____ Place of birth: _____
 Tax code: _____ Resident in: _____
 Street: _____ Requesting doctor/facility: _____

LABORATORY TESTS REQUESTED

Sampling date: _____

☐ PAPP-A and FREE BETA-hCG (assays only)

☐ BI-TEST (combined screening with risk calculation)

ULTRASOUND DATA

Maternal weight on sampling date: _____

_____ kg

Ethnic origin:

☐ Caucasian

☐ African

☐ Asian

☐ Other

No. of fetuses:

☐ 1

☐ 2

Pregnancy:

☐ Spontaneous

☐ IVF

Egg donation:

☐ NO

☐ YES:

Donor's age/year of birth:

Frozen embryo transfer:

☐ NO

☐ YES:

Freezing date:

Last menstrual period: _____

Ultrasound date: _____

CRL: _____ mm

Nuchal translucency NT: _____ mm

Nasal bone

☐ Present

☐ Absent

optional

Tricuspid regurgitation

☐ Present

☐ Absent

Abnormal ductus venosus flow

☐ Present

☐ Absent

MEDICAL HISTORY

Smoker:

☐ NO

☐ YES

Insulin-dependent diabetes:

☐ NO

☐ YES

Previous children affected by chromosomal aneuploidies

☐ NO

☐ YES:

☐ T21

☐ T13/T18

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I declare that I have read and understood the attached information leaflet and that I have understood that first trimester combined screening is not a diagnostic test but a probabilistic screening test, the result of which does not rule out the possibility that the fetus has Down syndrome or other chromosomal abnormalities.

I certify that I have been given clear and comprehensive explanations regarding the test, as I requested.

I decide ☐ **To consent** ☐ **Not to consent** to undergo the test of my own free will and with full awareness.

Date _____

Physician's signature _____

Patient's signature _____

Information guide to first trimester combined screening

First trimester combined screening is a probabilistic test that assesses the risk that the fetus is affected by a chromosomal trisomy of chromosomes 21, 13 and 18, which cause Down, Patau and Edwards syndromes respectively.

The statistical risk is calculated by dedicated software (Ssdwlab), recognised by the Fetal Medicine Foundation, which combines the patient's clinical parameters (such as age and weight), ultrasound markers detected during the scan (such as CRL and NT) and the measurement of two hormones (free β hCG, PAPP-A) in the maternal blood.

The test is performed between the 11th and the 13th+6 week of pregnancy, is non-invasive for the fetus and requires an ultrasound scan and a blood sample.

First trimester combined screening does not make any diagnosis, but assesses the risk (in terms of probability) that the fetus is affected by individual chromosomal abnormalities and syndromes. The probability is expressed as a numerical value (for example, 1:1000 means that, in this pregnancy, the probability that the fetus is affected by trisomy 21 is 1 in 1000).

A low-risk result indicates a lower risk than expected for maternal age; however, this does not mean that the fetus will certainly be "healthy", but that it is very unlikely to be affected by certain chromosomal abnormalities.

A high-risk result does not mean that the fetus is "ill", but only that it falls within an increased risk band for congenital anomalies, and further investigations are therefore necessary, to be assessed together with the attending Gynaecologist.

Storico delle revisioni

Rev.	Data	Descrizione	Redatto	Approvato
00	28/09/2026	Prima emissione. Traduzione in lingua inglese di TD-SGI-DEX010_00 (codifica provvisoria, da confermare in fase di registrazione in REG006).	RSGI	DG